

Mercy General Hospital

123 Main St, Springfield IL 62701 · (217) 555-0143

Patient: **Doe, Jane R.** · MRN: **00831742** · DOB: 1957-03-14 (68F)
· Sex: Female**HOSPITAL DISCHARGE SUMMARY**

Encounter date: 2026-04-22

Admission / Discharge

Admitted 2026-04-18 with acute decompensated heart failure. Discharged 2026-04-22 to home. Attending: Sarah Chen, MD (Cardiology).

Reason for Admission

68-year-old woman with HFrEF (LVEF 30%, ischemic), persistent atrial fibrillation, CAD s/p PCI to LAD in 2022, hypertension, type 2 diabetes, and CKD stage 3 presented with 5 days of progressive dyspnea on exertion, orthopnea, and 8-lb weight gain. She was diuresed with IV furosemide and transitioned back to oral therapy prior to discharge.

Past Medical History

- Heart failure with reduced ejection fraction (HFrEF), LVEF 30% (ischemic)
- Coronary artery disease, s/p PCI to LAD with DES, 2022
- Atrial fibrillation, persistent, CHA₂DS₂-VASc score 5
- Hypertension
- Hyperlipidemia
- Type 2 diabetes mellitus, on metformin
- Chronic kidney disease, stage 3 (baseline eGFR ~45 mL/min/1.73m²)

Past Surgical History

Appendectomy (1985). PCI to LAD with drug-eluting stent (2022).

Family History

Father: MI at age 64. Mother: type 2 diabetes, stroke at age 78.

Social History

Lives with husband. Former smoker, 20 pack-years, quit 2018. No alcohol. No illicit drug use. Walks 1 block before stopping for dyspnea.

Allergies

No known drug allergies.

Discharge Medications

1. Lisinopril 20 mg PO daily
2. Metoprolol succinate 50 mg PO daily
3. Furosemide 40 mg PO daily
4. Apixaban 5 mg PO BID
5. Atorvastatin 40 mg PO daily
6. Aspirin 81 mg PO daily
7. Metformin 1000 mg PO BID

Vitals at Discharge

BP 142/82 mm Hg · HR 76 (irregular) · RR 16 · SpO₂ 96% on room air · Weight 72.4 kg (dry weight 70.0 kg).

Pertinent Labs (most recent during admission)

Sodium 138 · Potassium 4.2 · Chloride 102 · CO₂ 24 · BUN 28 · Creatinine 1.4 · eGFR 45 mL/min/1.73m² · Glucose 168
Hemoglobin 11.8 · WBC 6.8 · Platelets 218
LDL-C 95 mg/dL · HDL 38 · Triglycerides 162 · Total cholesterol 168
HbA1c 7.8% · NT-proBNP 4,820 pg/mL · Troponin I peak 0.04 ng/mL

Imaging / Procedures

TTE 2026-04-19: LVEF 30%, severe global LV systolic dysfunction, moderate functional mitral regurgitation. See separate report.

Coronary angiography 2022: 90% mid-LAD stenosis, treated with DES.

Hospital Course

Patient achieved net diuresis of 4.2 L over four days with IV furosemide, then transitioned to her home oral dose.

Telemetry showed persistent AFib with controlled ventricular response on metoprolol. Apixaban was continued throughout the stay. No ischemic events on telemetry. Discharged in compensated state, ambulating without supplemental oxygen.

Discharge Diagnoses

1. Acute decompensated heart failure with reduced ejection fraction (HFrEF)
2. Persistent atrial fibrillation, anticoagulated
3. Coronary artery disease, s/p PCI to LAD 2022
4. Hypertension
5. Hyperlipidemia
6. Type 2 diabetes mellitus
7. Chronic kidney disease, stage 3

Follow-Up

Cardiology clinic in 1 week (Dr. Chen). Primary care in 2 weeks. Daily weights; call if weight gain >3 lb in 2 days.

Electronically signed: Sarah Chen, MD — 2026-04-22 14:21 CT